



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هبه رجب محمد احمد

المحافظة : القليوبية الحي / المدينة : شبرا الخيمة اول العنوان : طريق بيجام 46- 3 - ش الازهري

رقم البطاقة : 28103250101847 رقم التليفون : 01228217251 تاريخ تسجيل الإستمارة : 21-09-2026 العمر (تاريخ الميلاد) : 25-03-1981 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك الإخصائي الاجتماعي التوقيع: (-----) المقر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: ----- التوقيع: -----
الاسم: د/ندى حسن الوظيفة: ----- التوقيع: -----
الاسم: د/نرمين رمضان الوظيفة: ----- التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة. تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : ----- قسم : -----
جهة العلاج : ----- نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هبه رجب محمد احمد National ID: 28103250101847	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



١١ ٨٠/١١/٨٨٠٨



٨٢٧١٠١٠٥٨٨٠١٧٨ ١١/٠٨٠٨



DMB
کتابی
کتابی

جمهورية مصر العربية
بغافه تحقيق الشخصية

هبة
رجب محمد احمد
طريق بيجام ٤٦-٣- ش الازهرى
شبرا الخيمة اول - القليوبية

١٩٨١/٠٨
٢٨١٠٣٢٥٠١٠١٨٤٧
IQ1381661

جوسناف
روستاق
مستشفى

أ. د. ه. ه. ه.
أ. ه. ه.



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية العربية السورية
مستشفى

08/18/2026

MDT FORM

Name: هبة رجب محمد أحمد	MRN: 12945
Age: 45 Y	ID: 28103250101847
Menopausal Sta: PRE	Phone: 01228217251
NRS SCORE:	Marital Status: DIVORCED
BMI:	Family History: +VE FH MOTHER
Gender:	Reason Of MDT: NEW CASE
CO Morbidity: M. FEE	ECOG Performance Status: 0 0 1 0 2 0 3 0 4
Surgical History: RT PARA UMBILICAL HERNIA REPAIR 2014 SLEEVE GASTRECTOMY 2018	

Clinical Data:

BX: LT BREAST IDC G3
ER: 0/8 PR: 0/8 HER2: 0 KI67: 80%



History:

SM>>ACR C
LT.UIQ 11 COCK ZONE 2 MASS (2.1*1.9)MM& 2 SURROUNDING
SATELLITES WHOLE AREA 3.4*1.8 CM
ONE PATHOLOGICAL AXILLARY LN 2*7 MM CORTICAL THICKNESS 6 MM
BIRADS 5

Sonomammography:

CEM >>UIQ MASS ENHANCEMENT ALONG WITH INTIMATELY RELATED 2
CM ANTERIOR LINEAR NODULAR NON MASS ENHANCEMENT MEASURING
4.7*2.7*305 CM 8 CM FROM NIPPLE 0.6 CM FROM OVER LYING SKIN
SOLITARY PATHOLOGICAL AXILLARY LN 2*1.3 CM CORTICAL THICKNESS
0.5 CM BIRADS 6

Biomarker/Molecular

Mets Workup:

CT CAP >> FREE

Recommendations:

FOR POSSIBILITY OF CLIPPING THEN NACT THEN LT BCS & SLNB

MDT Outcome & Plan

FOR clip bracketing of the area
neoadj taxoi carbo 12 weeks with keytruda then AC*4 with keytruda then
LT BCS & SLNB

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

CT2N0M0

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. هبة رجب	د. هبة رجب		

Patient's Name: Heba Ragab Mohammed Ahmed
Date: 2026-07-08
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Left breast lump.

Findings:

Right breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with mottled appearance yet no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- Left UIQ speculated high density mass lesion with related two small lesions ,the whole area of affection measures 3.5 x 1.8 cm , no suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with mottled appearance.
- 11 o'clock, zone 2 , irregular spiculated h hypoechoic solid mass measures (21x 19 mm),two surrounding sateelites,the whole area of affectiona measures (3.4 x 1.8 cm).
- Normal skin/nipple/areola complex.
- One pathological level 1 left axillary LN ,it measures (20 x 7 mm) with thick cortex of 6 mm .

Conclusion:-

- **Right breast (BIRADS 1): Normal y.**
- **Left breast (BIRADS 5): 11 o'clock multifocal suspicious mass lesion with ipsilateral one pathological level 1 axillary lymph nodes .**

Recommendation:-

- **For biopsy and histopathological correlation.**

Prof.MONA ZAKY

Patient Name	هبة رجب محمد	Case No.	341807
Sex/Age	Female 45 Y	Patient No.	24726
Registered Date	13/7/2026 01:19:22 PM	Referred By Dr.	Interventional Radiology
Validation Date	02/8/2026 02:35:57 PM	Branch	Gustave Roussy
Reporting Date	03/8/2026 10:10:07 AM	Pathology No.	2199/26

Clinical Data: Left breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/07/2026, at 1:19 pm. Cold ischemia time is 20 seconds.

Gross Examination:

-Four fragmented grey, white tissue cores, ranging in size between 0.8 cm & 1 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type:** Invasive duct carcinoma, focally necrotized.
- **Histologic grade:** 3(differentiation 3, pleomorphism 3, mitosis 2)
- **TILS:** Moderate
- **DCIS:** Absent.
- **Microcalcification:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 80%.

DIAGNOSIS



Patient Name	هبة رجب محمد	Case No.	341807
Sex/Age	Female 45 Y	Patient No.	24726
Registered Date	13/7/2026 01:19:22 PM	Referred By Dr.	Interventional Radiology
Validation Date	02/8/2026 02:35:57 PM	Branch	Gustave Roussy
Reporting Date	03/8/2026 10:10:07 AM	Pathology No.	2199/26

Left breast mass, Tru-cut biopsy:

-Invasive duct carcinoma, Grade 3.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0)
- Ki67%: 80%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim





Patient ID	659304-6	Request Date	10-08-2026 02:06:17 pm
Acc.No.	626136669	Print Date	10-08-2026
Patient Name	السيدة / هبة رجب محمد أحمد	Sex / Age	Female / 45 Y
Referred by	د. /		
ID Number			

CLINICAL CHEMISTRY UNIT

Test	Result	Unit	Reference Range
RENAL PROFILE:			
Serum Urea	39	mg/dL	15 - 40
Serum Creatinine	0.7	mg/dL	0.5 - 1.2
Serum Uric Acid	2.9	mg/dL	2.9 - 6.6

Signature
Hagar Zien

تواصل معنا
مكان التردد وشبكة المختبر
SCAN CODE TO GET OFFER
@ezzlabor
01200555562



hotline 15032



اسم المريض بالإنجليزية: heba ragab mohamed	كود المريض: 28103250101847
تاريخ التقرير: 2026/08/12 09:53:13 AM	السن: ٤٥ سنة, ٤ شهر

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: 45 years old, left breast pathologically proven invasive ductal carcinoma grade III (-ve ER, -ve PR, -ve HER2 and Ki67= 80 %)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Minimal background enhancement.

ACR: C (heterogeneous dense)

Left breast:

- * **Low energy (Non contrast) images:** UIQ posterior third irregular shaped dense mass lesion with partially spiculated margin along with adjacent nodules
- * No suspicious microcalcifications
- * **Recombined (Post contrast) images** show corresponding
 - o Heterogeneous moderate conspicuity mass enhancement along with intimately related 2cm anterior linear nodular non-mass enhancement, the total enhanced process measuring about 4.7 x 2.7 x 3.5 cm along maximum diameters, being about 8 cm from the nipple and 0.6 cm from the overlying skin (at 11:00 axis)
 - o Normal skin and nipple areola complex
 - o Solitary pathological axillary lymph node seen at level I with cortical thickening, largest one measuring about 2 x 1.3 cm with irregular thickened cortex reaching 0.5cm

Right breast:

- * **Low energy (Non contrast) images** No asymmetry or distortion or suspicious microcalcifications.
- * **Recombined (Post contrast) images:** no pathological mass or non-mass enhancement.
- * Normal skin / nipple / areolar complex.
- * Nonspecific axillary lymph nodes

IMPRESSION:

- * **LEFT BREAST:** (BIRADS 6= pathological proven) UIQ pathologically enhancing mass with small anterior non-mass enhancement as described.
- * **RIGHT BREAST:** (BIRADS 1) No pathological uptake

Best regards,

Dr. EMAN ALI

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION SYSTEM AND DIGITAL
TRANSFORMATION



الهيئة العامة للتأمين الصحي
الإدارة المركزية لتنظيم المعلومات و التحول
الرقمي

رسوم الاستعلام 20 جنية

جهة الاصدار : فرع القليوبية

تاريخ الاصدار : 2026-8-5

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للالتحاق الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والارامل " و كذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسي " وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .
وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية
وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي
والبحث تبين أن

المسيد (ة) هبة رجب محمد بطاقة رقم 28103250101847

تليفون رقم: 01228217251

وطبقا الى فصل او استقاله او انتهاء مدة خدمة
يعتبر من الفئات غير المنتفعة بالتأمين الصحي صدر هذا الخطاب دون ادنى مسئولية على الهيئة العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الطباعة : 2026-8-5

الموظف المختص

سهام عبد الشكي

إفاداة مركز تنظيم المعلومات



مسلسل 2042420

الخطاب ساري لمدة ثلاثة أشهر من تاريخ الإصدار إلا في حالة تغيير حالة المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو وزارة التضامن الاجتماعي أو وزارة التربية والتعليم

الإدارة المركزية لتنظيم المعلومات و التحول الرقمي

049866

رقم الاصل

Patient's Name: Heba Ragab Mohammed Ahmed
Date: 2026-08-11
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE CT EXAMINATIUN OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- An enhancing neoplastic mass lesion is seen in the upper inner quadrant of the left breast and measures about 24x19 mm in dimensions with central area of breaking down and /or cystic degeneration. Another similar rather smaller lesion is also seen ,more interne to the former one and measures about 18x16mm. Each also shows an irregular outlines with foci of microcalcifications scattering at the periphery of the former lesion. Another tiny well-defined density is also seen in the same quadrant,at a higher level of the fore-mentioned ones and measures about 7.1 mm.The overlying skin is thickened,more externally and measures about 8.1 mm.
- Pathologically -enlarged left axillary lymph nodes are seen with loss of their hilar fat contents and the largest one measures about 25x16mm in dimensions.
- **Normal right breast.**
- **Benign-looking right axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 14 mm.**
- Hyperinflated both lungs.
- No sizable abnormally-enhancing pulmonary nodules ,other patchy areas of consolidations or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions.
- Average size of the heart with normal CT of its big vessels.
- Normal bones of the thoracic cage apart from mild spondylo-degenerative changes of the scanned spine.
- Border line size of the liver, measuring about 15.5cm ,showing diffuse reduced parenchymal attenuation and a homogenous enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.1 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.

Patient's Name: Heba Ragab Mohammed Ahmed
Date: 2026-08-11
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- No intra-vesical stones, solid masses, wall thickening or diverticulosis seen.
- A small right ovarian cyst is seen and measures about 36x31mm in dimensions.
- The rest of the pelvic structures are unremarkable.
- A right para-umbilical hernia is seen, measuring 9.5x3.4cm in dimensions, with its orifice=2.4cm and contains an omental fat.

IMPRESSIONS:

- Multifocal neoplastic lesions in the left breast. The overlying skin is thickened, more externally and pathologically -enlarged left axillary lymph nodes.
- Fatty infiltration of the liver with no focal lesions.

Prof/Hanan El-Ahwal.

ECHO REPORT

NAME: Mr. Ali and

ID NUMBER: /

DATE: 6 / 9 / 2026

SEX: /

AGE: /

EF	64%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVEDD	2.3	2-4cm	LA	3.5	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function Normal no significant ectopics

SWMA Normal

LA and Ao Root Dimensions Normal

Mitral Valve Normal

Aortic Valve Normal

Tricuspid Valve Normal

Pulmonary Valve Normal

Right Side and Systolic Function Normal

Intracardiac Masses and Effusion Normal

DR: Ali

مركز الأورام
Patient Name : دة رجب محمد
Referring Physician: Prof. Dr.

Date: 29/08/2026

Whole body PET/CT

Technique:-

Approximately one hour following the IV injection of the radiopharmaceutical, a total body PET – CT was acquired from mid thigh to base of skull with the arms extended above the head. CT was acquired for the purpose of anatomic localization & attenuation correction. Data was processed & displayed according to departmental protocol.

Report:

- Hepatic Reference: max SUV ~ 6.7.

Head & Neck:

- The brain exhibited normal FDG bio-distribution.
- Physiologic FDG uptake is seen in the oropharynx, salivary glands & larynx.
- No metabolically active FDG avid cervical LNs.
- No focal diffuse thyroid gland FDG uptake could be elicited.

Chest:

- No metabolically active FDG avid mediastinal or hilar lymph nodes are seen.
- No metabolically active FDG avid pulmonary masses .
- There are no pleural or pericardial abnormalities.
- Physiologic FDG uptake is noted in the myocardium.

Breast & axilla:

- Metabolically active FDG avid left sided breast , inner quadrant neoplastic mass, with enhanced parenchymatous density. This mass is seen with central breaking down (measuring 3.1x2.6cm) with max SUV~ 13.7) for follow up.
- Bilateral axillary few non FDG avid preserved hilar L.Ns (up to 1.2cm) for follow up.
- Normal appearance of the right breast.
- Clear both axilla.

مرضى الأورام
Patient Name : م.ه رجب محمد
Referring Physician: Prof. Dr.

Date: 29/08/2026

Abdomen & Pelvis:

- Mild hepatomegaly with normal FDG uptake & no metabolically active FDG avid focal lesions.
- The spleen is average in size showing normal FDG uptake with no metabolically active FDG avid splenic focal lesion.
- Metabolically inactive subcentimetric abdominal, pelvic L.Ns (reactive).
- Evidence of gastric sleeve.
- Right sided para umbilical hernia, with omental fat content.
- No metabolically active lymph nodes are seen.
- Physiologic FDG excretion is seen in the kidneys & bladder.
- No evidence of ascites.

Musculoskeletal:

- Normal FDG activity is seen along the rest of the surveyed skeleton. No sclerotic or lytic lesions are noted on CT.

OPINION:

- Positive PET/CT study shows:
- Metabolically active FDG avid left breast UIQ focal neoplastic masses for follow up.
- No metabolically active FDG avid nodal or distant metastases.

PET/CT Unit

Nuclear Medicine Department

DR/Omnia Talaat /, MD

Radiology Department

DR Heba Nabil /, MD

Revised by

DR/Mohamed Emam

ECHO REPORT

NAME: Mr. Ali and

ID NUMBER: /

DATE: 6 / 9 / 2026

SEX: /

AGE: /

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DR: Ali

مركز الأورام
Patient Name : دة رجب محمد
Referring Physician: Prof. Dr.

مستشفى ناصر
Date: 29/08/2026
Nasser Institute Hospital

خطاب الأورام

Whole body PET/CT

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Date: 29/08/2026

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PET/CT Unit

Nuclear Medicine Department

DR/Omnia Talaat /, MD

Radiology Department

DR Heba Nabil /, MD

Revised by

DR/Mohamed Emam

اسم المريض - المختبر
مختبر الشيخ دينا مسير محمد عطيه
مختبر الوحدة ا.د رانيا درويش
طبيب الوحدة ا.د. عطيه النش

رئيس مجلس الادارة
ا.د. مؤمنه كامل
مختبر المعامل
ا.د. هند الشربيني

PATIENT NAME هبة رجب محمد			Registered 24-08-2026 18:24:07	Collected 24-08-2026 18:37:25
Visit Number 68526514519	Age 45 Year	Gender Female	Authenticated 19-09-2026 13:10:15	Printed 20-09-2026 20:37:23
Referred By Prof : -			Client ID 55452	

MOLECULAR BIOLOGY REPORT

BRCA 1,2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result : No Pathogenic variants detected in BRCA1 or BRCA2 genes sequencing.

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the BRCA1 & BRCA2 genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.
- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, BRCA2 carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay
- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of BRCA1 & BRCA2 genes.
- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.

- Benign and likely benign variants are not included in this report.

- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.

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مدير الوحدة ا.د رانيا درويش
طبيب الوحدة ا.د. عطاء النش

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- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.
- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

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